

SUPERIOR COURT OF JUSTICE

MECIT DOĞAN

Plaintiff

Rebecca Nelson, for the Plaintiff

- and -

LESTER PAKULSKI, WANDA PAKULSKI,
MARGARET PAKULSKI and JOANNE
PAKULSKI, also known as JOANNE KALAJ

Defendants all unrepresented

Defendants

HEARD: April 18, 19 and 20, 2007

NEWBOULD J.

REASONS FOR JUDGMENT

[1] On April 6, 2003 the plaintiff fell at the rear of 8 High Park Boulevard in Toronto, where he rented a room. The house was owned by the defendants Lester and Wanda Pakulski. The plaintiff hit his head on the side of the building. He was taken to hospital. He claims damages for alleged negligence resulting from the failure of the owners to have the walkway cleared of snow and ice which he alleges caused him to fall.

[2] The defendants Margaret Pakulski and Joanne Kalaj are daughters of the owners. They were added as defendants in November 2004 arising from their conduct following the plaintiff's

fall. They were noted in default on January 7, 2005. They took no steps to later set that aside or defend the action against them.

[3] Mr. Dogan is 73 years of age. He was born in Yugoslavia and came to Canada in 1969. He last worked in 1985 when he went on long-term disability because of his heart condition.

[4] Prior to April 7, 2003, the date of the fall, Mr. Dogan had a number of medical problems. Dr. Vasie Stefou is a family practitioner. Mr. Dogan has been his patient since 1989. Dr. Stefou said that for the three years prior to the fall, Mr. Dogan had atrial fibrillation, a heart condition that results in less than full volume of blood going to the heart. This condition causes tiredness and weakness. Mr. Dogan also had what Dr. Stefou described as uncontrollable type 2 diabetes. He had been on medication for the diabetes for a long time. Dr. Stefou said that the diabetes can cause a loss of strength in a person's legs. Mr. Dogan also had high blood pressure that was poorly controlled. He had low back pain and was taking medicine for that.

[5] At the time of the fall, Mr. Dogan was renting a room in the basement at 8 High Park Boulevard. It was a large house that had been converted many years before into a number of apartments. All of the rented premises other than Mr. Dogan's room were on the first or second floor and the tenants used the front door of the property to gain entrance to their units. Mr. Dogan's entrance to his room was through a door at the back of the house. He had a small room. He had the use of a hot plate and refrigerator in the basement outside of his room. There was a laundry room in the basement, and a washroom. Other tenants who wanted to use the laundry room had to go outside of the building and access it through the door at the rear.

[6] The driveway between 6 High Park Boulevard and 8 High Park Boulevard was paved, as was the rear of the portion of the back of 8 High Park Boulevard leading to the back door. The back door was roughly in the middle in the back of the house and was covered by an overhang. To get to his room, Mr. Dogan had to walk along the driveway and then turn left and walk along the back of the house to the entrance.

Events of April 7 – 10, 2003

[7] In early April 2003 there was an unusual storm in Toronto. On April 3, 2003 there was 19.8 millimeters of precipitation consisting of freezing rain and ice pellets. On April 4 there was a total of 15.8 millimeters of precipitation consisting of freezing rain and ice pellets. On April 5 there was a total of 3.6 millimeters of precipitation consisting of freezing rain, ice pellets and snow. There was no precipitation on April 6. There was a total of 2.4 millimeters of precipitation on April 7 consisting of snow.

[8] A number of photographs were taken after the fall by neighbours who provided them to Mr. Dogan four or five days later. These show the side of the joint driveway at 6 High Park Boulevard having been cleared of snow. The side of the driveway at 8 High Park Boulevard was not cleared and the back of the house was not cleared of snow except for the portion under the overhang by the door. The snow removal at 8 High Park Boulevard was sporadic. According to Joanne Kalaj, sometimes she or her sister or brothers were involved and at one point, although she did not know in what year, someone was hired on a cash basis to remove snow from time to time. She said that her parents have five or six rental properties and that the sidewalks needed to be cleaned. I do not accept her assertion at trial that Mr. Dogan had agreed at the outset of his tenancy that he was to clear the walkway. That is inconsistent with her discovery evidence, the discovery evidence of Margaret Pakulski, and inconsistent with how the property was maintained.

[9] The photograph of the rear entrance to the house shows a bag of salt leaning against the wall under the overhang. Mr. Dogan said that that bag of salt was not there on the day of the fall but it was there when he came back from the hospital the following day. He said he had never seen a salt bag in the back before although sometimes there was salt put down. There was no evidence that the salt bag had been in that spot before his fall.

[10] Mr. Dogan said that when he came home the night before the fall, there was ice rain and it was snowing. That is inconsistent with the records of Environment Canada that indicate that it was mainly clear on April 6, 2003.

[11] Mr. Dogan said that on the day of the fall, he was going out to a restaurant. He thought it was late in the afternoon, perhaps 5:00 to 6:00 p.m. He said that when he was walking along the back of the house, there was ice under the snow. He said he was wearing snow boots. His right leg slipped and he fell on his right side and hit his head on a column of the wall that protrudes a little from the wall. Mr. Dogan said that after the fall, an ambulance came to get him. He said that the neighbour's daughter called the ambulance and he is not sure how long it took to arrive. The ambulance call report indicates that prior to the crew's arrival, a neighbour had wrapped Mr. Dogan in a blanket. The ambulance call report and the emergency department records at the St. Joseph's Health Care Centre state that Mr. Dogan called 911 on his cell telephone. It is possible that the neighbour may have used Mr. Dogan's cell phone to call the ambulance, but no neighbour was called as a witness. If Mr. Dogan called himself, presumably he called right after the fall. If the neighbour called, there is no evidence how long after the fall the neighbour arrived. The ambulance crew was notified at 7:24 p.m. and arrived on the scene at 7:38 p.m.

[12] The information on the ambulance call report states that Mr. Dogan was conscious and alert when the ambulance crew arrived and complained of headaches, dizziness and lower back pain. The ambulance call report states that there was an abrasion and contusion to his right parietal area. Once he was moved into the ambulance, his level of awareness decreased rapidly and he became verbally unresponsive and no longer responded to commands. The hospital records indicate that he was not initially responsive when first seen in the emergency department although by the following day he was fully alert. The emergency records indicate a complaint of a head injury.

[13] It is apparent that Mr. Dogan has a history of drinking, and an issue arises as to whether he was intoxicated at the time of his fall. There is in the record an emergency out patient record at St. Joseph's Health Centre dated May 20, 2001 dealing with Mr. Dogan's complaints of leg weakness at that time. There is a note "Hx alcoholism" which I take to be a reference to a history of alcoholism. The report indicates an admission diagnosis as possibly related to ETOH use, which I take to be a reference to Ethanol or ethyl alcohol use.

[14] Mr. Gurney Puddle gave evidence on behalf of the defendants. He has lived in the house since 1988. There clearly is no love lost between Mr. Puddle and Mr. Dogan. Mr. Puddle said that Mr. Dogan is intoxicated every day of the week and that he sees him at the bar every day. He asserted that he saw Mr. Dogan at a bar at 5:00 p.m. on the day of the fall, and was not surprised to later hear that he had fallen. He did not say what amount, if any, he had seen Mr. Dogan drink that day. He was not cross-examined. Mr. Dogan did not give any reply evidence denying that he was at a bar at 5:00 p.m. on the day of the fall. I am left with some questions as to whether Mr. Dogan was in fact at a bar at 5:00 p.m., and, if so, what he had to drink at the time.

[15] The ambulance call report indicates that a number of checks of various kinds were made on Mr. Dogan. There is no reference of any kind to any signs of alcohol use by Mr. Dogan. The emergency out patient record for that day also contains no evidence of alcohol use. Blood tests were ordered, including a toxic profile. The laboratory report from the hospital indicates that the blood for the testing was taken at 8:54 p.m. that day. The level of Ethanol was less than 2 mmol/L and the reference range, which I take to be the norm, was stated to be less than 39 mmol/L. The report did not indicate that the reading of Ethanol was either high or low. Because it is not clear exactly when the fall occurred, it is not clear how much time elapsed after the fall before the blood sample was taken or the effect of the passage of time on the Ethanol levels in Mr. Dogan's blood.

[16] On the evidence before me, I cannot find that Mr. Dogan was intoxicated at the time of the fall. The most that can be said is that he may have had something to drink that day.

Liability

[17] A landlord's obligations to his tenant at common with respect to the means of access to the premises was described by Professor Di Castri in his book, *Occupiers' of Liability*, Burroughs & Co. 1980, at page 150 as follows:

A landlord's implied contractual obligation to his tenant in respect to the means of access under his control to the premises leased is to use reasonable care so that the

means of access do not become unsafe; the obligation is wider than merely safeguarding the tenant from the risk of a concealed danger or trap.

The obligation is based on contract, express or implied although the test of liability is one of pure negligence: did the landlord take reasonable care to ensure that the means of access did not become unsafe?

A tenant's knowledge of the defect goes to contributory negligence only ...

[18] See also *Sinclair v. Hudson Cole & Fuel Oil Ltd.*, [1966] 2 O.R. 256 and *Williams & Rhodes, Canadian Law of Landlord and Tenant*, para. 10: 4: 2 at pages 10-17.

[19] The obligations of the landlord as an occupier of property is now dealt with in the *Occupier's Liability Act*, R.S.O. 1990, Chapter 0.2. The relevant provisions of that *Act* are as follows:

1. In this *Act*,
“occupier” includes,
 - a) a person who is in physical possession of premises, or
 - b) a person who has responsibility for and control over the condition of premises or the activities there carried on, or control over persons allowed to enter the premises.
2. Subject to section 9, this *Act* applies in place of the rules of the common law that determine the care that the occupier of premises at common law is required to show for the purpose of determining the occupier's liability in law in respect of dangers to persons entering on the premises or the property brought on the premises by those persons. R.S.O. 1990, c. 0.2, s. 2.
3. (1) An occupier of premises owes a duty to take such care as in all the circumstance of the case is reasonable to see that persons entering on the premises, and the property brought on the premises by those persons are reasonably safe while on the premises.
8. (1) Where premises are occupied or used by virtue of a tenancy under which the landlord is responsible for the maintenance or repair of the premises, it is the duty of the landlord to show towards any person or the property brought on the premises by those persons, the same duty of care in respect of dangers arising from any failure on the landlord's part in carrying out

the landlord's responsibility as is required by this Act to be shown by an occupier of the premises.

9. (3) The *Negligence Act* applies with respect to causes of action to which this Act applies. R.S.O. 1990, c. 0.2, s. 9.

[20] The *Occupiers' Liability Act* has been applied to landlords. See *Manning v. 3980 Investments Ltd.*, [2003] O.J. No. 1937 in which a visitor to an apartment building slipped on a patio stone walkway that was not properly maintained of its icy condition. See also *Johnston v. Standard Life Assurance Co.* (1990), 73 O.R. 2nd 495 involving a loose mat improperly maintained in the parking garage of an apartment building.

[21] In *Waldick v. Malcolm* (1990), 70 O.R. 2nd 717, aff'd [1991] 2 S. C. R. 456, Blair J.A. stated the following regarding an occupier's affirmative duty to take care to make the premises reasonably safe:

All courts have agreed that the section imposes on occupiers an affirmative duty to make the premises reasonably safe for persons entering them by taking reasonable care to protect such person from foreseeable harm. The section assimilates occupiers' liability with the modern law of negligence. The duty is not absolute and occupiers are not insurers liable for any damages suffered by persons entering their premises. Their responsibility is only to take "such care as in all the circumstances of the case is reasonable". The trier of fact in every case must determine what standard of care is reasonable and whether it has been met. Occupiers are also not liable in cases where the risk of injury is "willingly assumed" by persons entering the premises or to the extent that such persons are negligent. The nature and extent of these two exceptions will be examined later.

[22] *Waldick v. Malcolm* involved a plaintiff who slipped and fell on the driveway of the defendant's farm. The defendants had shoveled the steps and had salted the approach to the door but had not salted the driveway over which the plaintiff walked. The trial judge, Austin J., accepted that icy conditions during Ontario winters are a fact of life but held that there was an affirmative duty to see that visitors such as the plaintiff were reasonably safe.

[23] In my view Mr. and Mrs. Pakulski, the owners of 8 High Park Boulevard, were negligent. It was known that the plaintiff had to traverse the rear of the property from the doorway to the driveway in order to enter or leave his living quarters in the basement. The conditions at the

time Mr. Dogan fell were not of recent origin such that the defendants would not have had a reasonable time to see that the back area was cleared of snow and ice. The conditions began on April 3, 2003. There were two days of freezing rain and ice pellets of a substantial amount, one day of a small amount of freezing rain and ice pellets with some snow, one day with no precipitation and on the day of the fall 2.4 millimeters of snow. Steps should have been taken to clear the ice and snow long before the day of the fall. Even after the fall, when a bag of salt was put beside the back door, that was not sufficient. There was a positive obligation on the owners to see that the snow and ice was removed.

[24] I further find that the ice and snow conditions at the rear of the house caused Mr. Dogan to fall. There was some suggestion by witnesses that Mr. Dogan purposely fell in order to manufacture a claim. There is no evidence to support this assertion other than the assertion itself and I do not accept it. Mr. Dogan obviously injured himself and I am not prepared to accept that he purposely did so. I accept his evidence that his right leg slipped.

[25] Section 4(1) of the *Occupier's Liability Act* provides that the duty of care provided for in subsection 3(1) does not apply in respect of risks willingly assumed by the person who enters on the premises, but in that case the occupier owes a duty to the person to not act with reckless disregard of the presence of the person. That section has been narrowly interpreted to apply to situations only if the plaintiff has assumed both the physical and legal risks caused by a defendant's negligence. See *Waldick v. Malcolm, supra*. In this case there is no evidence that Mr. Dogan assumed such risks. In any event, had he done so, I would have found that Mr. and Mrs. Pakulski acted with reckless disregard of Mr. Dogan in failing to have the snow and ice removed in a timely fashion.

[26] It is apparent, however, that Mr. Dogan's own negligence contributed to the fall. *The Negligence Act*, R.S.O. 1990 ch. N.1, which is applicable, provides in these circumstance for an apportionment of the damages. Sections 3 and 4 provide:

3. In any action for damages that is founded upon the fault or negligence of the defendant if fault or negligence is found on the part of the plaintiff that contributed to the damages, the court shall apportion the damages in

proportion to the degree of fault or negligence found against the parties respectively. R.S.O. 1990, c. N.1, s. 3.

4. If it is not appropriate to determine the respective degree of fault or negligence as between any parties to an action, such parties shall be deemed to be equally at fault or negligent. R.S.O. 1990, c. N.1, s.4.

[27] Mr. Dogan had to be aware from his previous comings and goings since the onset of the storm on April 3, 2003 that the conditions at the back of the house over which he walked were hazardous. He said he was wearing snow boots, which would assist him, but it had to be obvious to him that he would have to take extra care and caution because of the conditions. While there was new fallen snow on the day of the fall of 2.4 millimeters, the ice under the snow that was present from April 3 was still there. Mr. Dogan should have taken extra care, including perhaps walking with one hand on the wall to help him steady himself, in order to avoid falling. Walking through snow and ice is not an unknown activity in this country.

[28] Advance knowledge of icy or dangerous conditions that caused persons to slip and fall have led to findings of contributory negligence. See, for example, *Manning v. 3980 Investments Ltd.*, *supra*, *Litwinenko v. Beaver Lumber Co.*, [2006] O.J. No. 3383, *Bogoroch v. Toronto*, [1991] O.J. No. 1032 and *Johnston v. Standard Life Assurance Co.*, *supra*. In *Waldick v. Malcolm*, *supra*, Austin J. refused to find contributory negligence on the part of the plaintiff who had previously walked across the icy driveway and knew its condition. Blair J.A. in the Court of Appeal in that case stated that it was quite possible that other judges might have reached a different conclusion on contributory negligence.

[29] Each case must be decided on its own facts. Mr. Dogan failed to take reasonable precautions to protect himself from the negligence of Mr. and Mrs. Pakulski and his negligence contributed to the damages.

[30] The respect of degree of fault between the plaintiff and Mr. and Mrs. Pakulski is difficult to assess, but I apportion the fault 40% to Mr. Dogan and 60% to Mr. and Mrs. Pakulski.

Medical Sequel

[31] At 9:08 p.m. on April 7, 2003 a CT Scan of Mr. Dogan's head was carried out. The result was normal. Dr. Gawel, a neurologist, said in evidence that the CT Scan meant that Mr. Dogan did not have any internal bleeding of the brain. The hospital records also indicated that cervical spine x-rays and facial bone x-rays taken of Mr. Dogan were also normal.

[32] The discharge report from the hospital on April 8, 2003 stated that Mr. Dogan did have a head injury. He appeared to be stable and was fit for discharge. He was cautioned to return to the hospital if he developed a worsening of his headache or any new symptoms. He was told that he could treat the dizziness symptomatically with Gravol.

[33] On April 10, 2003 Mr. Dogan went back to the St. Joseph's Health Care Centre. He complained of headaches and of double vision which he said had started the day before and said he saw black spots flying in front of his eyes. He complained of shaky legs. He remained in the emergency department until the following day when he was released.

[34] Dr. Stefou first saw Mr. Dogan after the fall on April 14, 2003. At that time Mr. Dogan said that he had felt dizziness and had headaches for approximately one week. Dr. Stefou examined him and noted that he was not in acute distress, that his head and neck were normal, that his chest was clear and his blood pressure was 160/80. He noted that Mr. Dogan should continue with the treatment that he had been given at the hospital.

[35] Mr. Dogan returned to Dr. Stefou on April 22, 2003. He complained of headaches, dizziness and double vision. Dr. Stefou checked his eyes and found nothing abnormal. Because of this new complaint, and of the prior hit to the head, Dr. Stefou referred him to Dr. Gawel, a consulting neurologist.

[36] Dr. Stefou's notes indicate that he again saw Mr. Dogan every two weeks to the end of 2003, on which occasions Mr. Dogan complained of headaches and dizziness. The visits have continued although for the last year and half they have been once a month. Dr. Gawel says that Mr. Dogan continues to complain of headaches and dizziness. The complaints of double vision are no longer made although Dr. Stefou is not sure when that complaint stopped.

[37] Mr. Dogan was first seen by Dr. Gawel on May 20, 2003. Dr. Gawel is a neurologist with impressive credentials. He holds positions at Sunnybrook and Women's College Health Sciences Centre and at the Scarborough Centenary Hospital. He is a founding member and president of the Canadian Headaches Society and a member of the board of the International Headache Society. He was an impressive witness as was Dr. Stefou. Both were objective.

[38] Dr. Gawel noted in his May 20, 2003 report to Dr. Stefou that Mr. Dogan was smoking ten cigarettes a day and drinking beer. One cannot overlook Mr. Dogan's habits. Dr. Stefou said in his evidence, as it is generally known, that type 2 diabetes can be controlled by weight loss, exercise and a cessation of smoking. Smoking and drinking could only negatively affect a person such as Mr. Dogan with his history of health problems.

[39] On May 20, 2003 Dr. Gawel examined Mr. Dogan. He reported normal cranial nerves, normal power and tone in the limbs but with a marked tremor when doing finger nose testing. He reported that Mr. Dogan's Hall pike manoeuvre is positive with the left ear down, which Dr. Gawel explained meant that the result was not good and indicated dizziness. Dr. Gawel said that it looked as though Mr. Dogan had caused damage to his left labyrinth, a part of his left ear. Dr. Gawel said that trauma was a possible cause of that damage but he could not state what in fact the cause was.

[40] Dr. Gawel arranged for an MRI to be done. The MRI was done on August 29, 2003 and Dr. Gawel reported its results to Dr. Stefou on September 9, 2003. He reported that the MRI was suggestive of demyelination, which means that his brain was not conducting impulses properly. His report stated that Mr. Dogan seemed quite unsteady and had some finger nose ataxia. Dr. Gawel said that that suggested a brain stem problem that could be caused by Multiple Sclerosis or a stroke or trauma or alcohol or a number of things. He also noted in his report some minimal nystagmus, which he described as a jerking of the eye when turned, suggesting a disturbance in input into the eye movement caused either by the brain or ear. Dr. Gawel said he was not too sure what to make of MRI report because it suggested that the demyelination might have been caused by MS or vascular changes in the brain.

[41] Dr. Stefou said that after Mr. Dogan was told of the potential MS diagnosis, he complained to Dr. Stefou of being anxious and depressed, and Dr. Stefou's notes indicate that that complaint was made on September 22, 2003. As a result, Dr. Stefou referred him to Dr. Ratnanather, a psychiatrist. Dr. Ratnanather reported on November 27, 2003 to Dr. Stefou that Mr. Dogan was going through a major depression as a result of hearing that he had MS and that he was obsessed with the thought and could not sleep at night. Dr. Ratnanather prescribed medication for the problem.

[42] Dr. Gawel again saw Mr. Dogan on January 19, 2004. He noted in his report to Dr. Stefou that Mr. Dogan told him that day that his memory was getting bad. He did a mini mental examination which Mr. Dogan scored at 27 out of 30, which Dr. Gawel said was not too bad. He prescribed medication for the memory problem and arranged for a further MRI.

[43] A further MRI was done on April 8, 2004. The findings were unchanged and Dr. Gawel said that the report was not of great significance to him. He was still uncertain whether results of the MRI indicated Mr. Dogan had suffered a small stroke or had MS.

[44] Dr. Gawel saw Mr. Dogan again on October 18, 2004. At that stage Mr. Dogan was complaining of feeling unsteady on his feet, getting electric like shocks going up his face and feeling dizzy. There is no indication that there were complaints to Dr. Gawel of headaches. Dr. Gawel's examination indicated that Mr. Dogan had trouble walking, had a very minimal finger nose ataxia and some positional dizziness. He noted in his report to Dr. Stefou that Mr. Dogan may have developed a further lacunar infarct possibly brain stem, and he said in evidence that the infarct could be an explanation for the dizziness and unsteadiness. He arranged for a further MRI.

[45] The further MRI was done on November 29, 2004. The MRI report, like the earlier MRI reports, noted white matter in the brain with a slight increase. Dr. Gawel's report to Dr. Stefou on this MRI is dated May 2, 2005. He stated that Mr. Dogan likely has MS as well as lacunar infarct. He said that the symptoms shown in the MRI could explain both diagnoses.

[46] Dr. Gawel saw Mr. Dogan on December 8, 2005. At that time he was not complaining of headaches but was complaining of unsteadiness. Dr. Gawel's report that day to Ms Nelson, counsel for the plaintiff, stated the following:

Since my report Mr. Dogan has had further problems in that he has developed frank diabetes, and pains in his chest. In September 2005 he was admitted to St. Joseph's Hospital for four days, and was investigated thoroughly, but apparently as he was leaving he found he could not walk, and was re-admitted for another eight days. He is now on a plethora of medications, ie. Pariet, Monacor, Norvasc, Propafenone, Reminyl, Altace, Aspirin, Lipitor and Cardura for his diabetes, blood pressure, cholesterol and memory problems.

On examination Mr. Dogan had problems with tandem gain; he is unsteady on his feet. He also had marked tremor, nystagmus in both directions of lateral gaze, and a cataract in his right eye; he had marked finger-nose and heel-shin ataxia, with normal sensation, brisk reflexes and down-going plantars. His neurological examination, therefore has deteriorated significantly, presumably from the spread of his lesions, which could either be Multiple Sclerosis or multiple infarcts. He requires a further MRI, which I have taken the liberty of arranging from a medical point of view. If you wish to expedite this, we could arrange this on a third party basis; nevertheless it is required.

[47] At the trial, Dr. Gawel said that he now thought it unlikely that Mr. Dogan had MS. He based this on the fact that with MS there tends to be continuing changes in lesions in the brain, and in Mr. Dogan's case it has turned out to be stable.

[48] In November of 2006 Dr. Stefou referred Mr. Dogan to another psychiatrist, Dr. D'Alessandro. He did so because Mr. Dogan was still complaining of anxiety and depression. Dr. Stefou wanted a second opinion. Mr. Dogan saw Dr. D'Alessandro on November 22, 2006. He prescribed different anti-depressant medication.

[49] The record contains a number of other reports and records of Mr. Dogan having been seen by various health care professionals at the St Joseph's Health Centre. On March 6, 2006 Dr. Choi, a cardiologist, reported that Mr. Dogan has significant engina. On January 25, 2007 Dr. Choi reported that Mr. Dogan had previously cut down on smoking but he had increased it again and referred to a number of problems, although he did state that Mr. Dogan continues to

feel well. He said that Mr. Dogan should be on beta blocker, but that he was not too keen on any further medications.

[50] Mr. Dogan's evidence at the trial was that he continues to suffer from dizziness and headaches. He says he cannot sleep. He gets nervous and angry and has difficulty controlling himself. He says that his double vision continues. He said that the medication was helping with his headaches, but when he gets angry they come back and that happens everyday. He said he has dizziness from any head movement and that he has double vision when looking either short or long distances. Mr. Dogan said prior to the accident he walked three to four hours per day and was very strong. He said after the accident he has now lost about 70% of the strength in his legs, which started three or four months after his accident. He said he does very little walking and his legs become numb and swollen. He said that on bright days his double vision is at its worst.

Causation

[51] The Supreme Court of Canada in *Athey v. Leonati*, [1996] 3 S.C.R. 458 dealt extensively with the general principles of causation in tort cases. I take from the reasons of Major J. the following propositions:

- (i) Causation is established where the plaintiff proves to the civil standard on a balance of probabilities that the defendant caused or contributed to the injury.
- (ii) The general, but not conclusive, test for causation is the "but for" test, which requires the plaintiff to show that the injury would not have occurred but for the negligence of the defendants.
- (iii) The "but for" test is unworkable in some circumstances, so the courts have recognized that causation is established where the defendant's negligence "materially contributed" to the occurrence of the injury. A contributing factor is material if it falls outside the *de minimis* range.
- (iv) The plaintiff must prove that the defendant's tortious conduct caused or contributed to the plaintiff's injury. The causation test is not to be applied too rigidly. Causation need not be determined by scientific precision. It is essentially a practical question of fact which can best be answered by ordinary common sense.

- (v) It is not now necessary, nor has it ever been, for the plaintiff to establish that the defendant's negligence was the sole cause of the injury.
- (vi) The law does not excuse a defendant from liability merely because other causal factors for which he is not responsible also helped produce the harm.
- (vii) The defendant need not put the plaintiff in a position better than his or her original position. The defendant is liable for the injuries caused, even if they are extreme, but need not compensate the plaintiff for any debilitating effects of the pre-existing condition which the plaintiff would have experienced anyway. The defendant is liable for the additional damage but not the pre-existing damage. Likewise, if there is a measurable risk that the pre-existing condition would have detrimentally affected the plaintiff in the future, regardless of the defendant's negligence, then this can be taken into account in reducing the overall award. This is consistent with the general rule that the plaintiff must be returned to the position he would have been in, with all of its attendant risks and shortcoming, and not a better position.

(a) **Headaches and Dizziness**

[52] Dr. Stefou expressed the opinion that the headaches and dizziness experienced by Mr. Dogan were caused by the fall. He bases that on the fact that there were no complaints of headaches and dizziness before the fall and there are no other causes that he has seen that would explain the headaches or dizziness. He doubts that the headaches will change in the future though he said they may get worse. He thinks that the dizziness will probably remain at the same level.

[53] Dr. Gawel said that one would expect headaches and dizziness after the head injury suffered by Mr. Dogan from the fall. He also expressed the opinion that the headaches and dizziness were likely caused by the head injuries suffered in the fall. He said that usually these improve after about six weeks, although in some cases it can last for several years.

[54] Dr. Gawel said that it was difficult to make a prognosis as to how long the dizziness would last. He said that people over 70 often have dizziness. If dizziness is caused by lesions on the brain or labyrinth problems in the ear, it is unlikely to improve. Dr. Gawel said he could not say whether the prolonging of the dizziness experienced by Mr. Dogan was caused by the fall or

other factors, and when asked whether he could state what the probabilities were as to whether the fall or other factors caused the prolonging of the dizziness, he said he could not.

[55] Based on the evidence, I find that the headaches and dizziness experienced by Mr. Dogan were caused by the fall on March 7, 2003. How long the headaches and dizziness have lasted is uncertain. While Mr. Dogan has apparently continued to complain of these, I gained the impression from Mr. Dogan that generally he was exaggerating his complaints. There is no objective scientific evidence of headaches and the last objective evidence of dizziness was seen in Dr. Gawel's testing done on October 18, 2004.

[56] With respect to the duration of the headaches, I find that that is due to the fall and not to other factors. Both Drs. Stefou and Gawel agreed there was no known reason for the headaches other than the fall.

[57] With respect to the duration of dizziness, I accept the evidence of Dr. Gawel that it is not known whether the continuation of dizziness was caused by the fall or by other factors relating to Mr. Dogan's health nor can it be said what the probabilities of either cause are. Dr. Gawel expressed the opinion that a lacunar infarct could be an explanation for dizziness and unsteadiness, and the possibility of such vascular changes was noted as early as September 9, 2003 from the first MRI. With the opinion of Dr. Gawel expressed at trial that Mr. Dogan likely does not have MS, the alternative diagnosis of lacunar infract is much more probable. Dr. Stefou said that both Mr. Dogan's heart condition and diabetes could cause a loss of strength and that some people refer to weakness caused by a heart condition as dizziness. I am left with the conclusion that whatever the duration of the dizziness, it may be the result of the fall or may be the result of other medical conditions.

(b) Double Vision

[58] The emergency outpatient record for April 10, 2003 states that Mr. Dogan that day told the hospital that he had experienced double vision from the day before. Mr. Dogan did not make that complaint when he first saw Dr. Stefou on April 14, 2003. Dr. Stefou's note of April 22,

2003, 15 days after the fall, indicates that Mr. Dogan that day first complained that he had double vision. That is one of the reasons why Dr. Stefou referred him to Dr. Gawel. Dr. Gawel's reports mentioned nothing of double vision. I was not referred to any other note of Dr. Stefou that referred to double vision. Although Mr. Dogan said in his evidence that he continues to have double vision problems, that is contradicted by Dr. Stefou's evidence that he no longer has such complaints.

[59] Dr. Gawel expressed the view that the effects of the fall would be experienced at the time and that conditions that arose later would not be caused by the fall. He stated that the deterioration that he described in his report of December 8, 2005 would not have been caused by the fall.

[60] Apart from the few references to double vision that I have referred to, there is no other evidence supporting a problem of double vision. I think that this is a complaint of Mr. Dogan that he is exaggerating. I am not prepared to find that he has suffered any serious double vision as a result of the fall that occurred on April 7, 2003. If he suffered any serious double vision, it was not caused by or contributed to by the fall.

(c) Depression and Anxiety

[61] Mr. Dogan complained of anxiety and depression to Dr. Stefou shortly after being informed of the diagnosis of Dr. Gawel that he may have MS. That was approximately 5½ months after the fall. It is clear that the diagnosis by Dr. Gawel of the possibility of MS was derived from the MRI reports which indicated to him either MS or vascular changes to the brain not related to the fall. It is argued on behalf of Mr. Dogan that the diagnosis of MS at the time was a mis-diagnosis and that the resulting depression was in some way caused directly or indirectly by the fall.

[62] I do not accept that submission. There is no basis for any conclusion that the diagnosis of Dr. Gawel was a mis-diagnosis. He diagnosed MS as a possibility, and it was only after further MRI tests and the passage of time in which the white matter disease did not significantly

progress that he was able to conclude by the time of the trial that Mr. Dogan likely did not have MS. Even if there had been a mis-diagnosis, that could not be said to be the result of the fall, and depression caused by a mis-diagnosis could not attract liability on the part of the defendants.

[63] Dr. Stefou expressed the view that Mr. Dogan's depression and anxiety was caused both by Mr. Dogan learning of the possible MS diagnosis and of the dizziness. Dr. Ratnanather, the psychiatrist to whom Mr. Dogan was referred for the condition, concluded that the depression was the result of Mr. Dogan hearing that he had MS, and I accept that opinion. The complaint of depression arose long after the fall, and only after the diagnosis of possible MS, and commonsense indicates the MS diagnosis as the cause. Dr. Gawel's opinion that conditions that arose later would not have been caused by the fall leads to the same conclusion.

[64] In the result, I find that the fall on April 7, 2003 was not the cause of, nor did it contribute to, the depression later experienced by Mr. Dogan.

(d) Weakness of Legs

[65] Mr. Dogan said that he used to walk four hours per day but that he now has lost about 70% of the strength in his legs, which condition, he said, started three or four months after his accident. He now says he does very little walking.

[66] It is apparent that Mr. Dogan had problems with leg weakness long before the fall. The emergency outpatient record at St. Joseph's Health Centre of May 22, 2001 concerned Mr. Dogan's complaints of leg weakness at that time, and referred to his history of alcoholism as being a possible cause.

[67] I am mindful of Dr. Gawel's opinion that one would expect the effects of the fault to be dealt with at the time and that later occurring problems would not in his view have been caused by the fall. Mr. Dogan did not make complaints about weak legs shortly after the fall and, if one were to accept his evidence, the problem only started three or four months afterwards.

[68] Dr. Stefou expressed the opinion that Mr. Dogan's diabetes and heart condition could cause weakness and loss of strength in his legs. Old age itself can cause weakness in the legs and, with the medical problems that Mr. Dogan had before the fall, and his lifestyle, including smoking and drinking, it is not surprising that he has a weakness or lack of strength in his legs.

[69] I cannot find that any weakness experienced by Mr. Dogan in his legs was caused, or contributed to, by the fall on April 7, 2003.

(e) Summary

[70] In summary, I find that Mr. Dogan suffered headaches and dizziness after the fall on April 7, 2003 that were caused by the fall. The length of time that Mr. Dogan experienced his headaches, or will experience them in the future, is uncertain.

[71] With respect to the dizziness, the length of time that Mr. Dogan experienced the dizziness, or will experience it in the future, is also uncertain. It cannot be said that the length of time that the dizziness has been experienced has been caused entirely by the fall. The length of the dizziness may have been caused by other health problems Mr. Dogan had and this possibility, according to *Athey v. Leonati, supra*, is one to be taken into account in assessing damages.

Damages

[72] Ms Nelson for Mr. Dogan provided me with a number of precedents for guidance as to what the measures of damages should be. The range in those cases was \$15,000 to \$50,000. She submits that a reasonable range for general damages in this case is \$30,000 to \$40,000.

[73] Each case, of course, depends upon its own facts. Taking into account Mr. Dogan's health problems that he has had regardless of the fall and the difficulty in determining the length of the headaches and dizziness that can be attributed to the fall, I assess general damages at \$15,000.

[74] Mr. and Mrs. Pakulski are therefore liable for \$9,000, being 60% of the damages.

[75] There is a subrogated claim by OHIP totaling \$7,651.27. It consists in large measure of all of the visits by Mr. Dogan to Dr. Stefou from April 2003 to April 2007. It also contains payments to St. Joseph's Health Centre and for Dr. Gawel and Sunnybrook. There is no evidence as to how these expenditures were compiled although Ms Nelson said that she requested OHIP to compile their expenses caused by Mr. Dogan's dizziness and headaches complaints.

[76] I am not prepared to accept the entire OHIP claim down to 2007. There is uncertainty as to the length of the headaches and dizziness caused by the fall and Mr. Dogan's MRI history has been caused in large part by the possibility of MS or other neurological problems. Mr. Dogan saw Dr. Stefou regularly long before the fall and that has continued to date. He has many medical problems that require regular visits to Dr. Stefou. The OHIP summary to the end of 2003 totals approximately \$2,000 and to the end of 2004 approximately another \$2,100, including \$1,370 for two further MRIs. In my view an appropriate claim by OHIP is \$1,500 and Mr. and Mrs. Pakulski are liable for 60% of that amount or \$900.

Actions of Margaret Pakulski and Joanne Kalaj

[77] This action was first commenced against only Lester and Wanda Pakulski on August 5, 2004. It was served at 123 Colbeck Street in Toronto on August 10, 2004. Margaret Pakulski lives at that address. She and her sister Joanne Kalaj became aware of the statement of claim at that time. It was Margaret Pakulski who prepared the statement of defence on behalf of her parents.

[78] In late August early September 2004, Joanne Kalaj met with Mr. Dogan. She had a document for him to sign and said she was going to give him \$500 if he signed it. Mr. Dogan told her that he had a lawyer and that she should talk to his lawyer.

[79] On the following day Joanne Kalaj, Margaret Pakulski and her boyfriend went to Mr. Dogan's premises at 9:00 p.m. They knocked at the door. Margaret swore filthy language. They again had an envelope and told Mr. Dogan that he had to sign the document. Mr. Dogan

again told them that he had a lawyer and they should talk to him. Mr. Dogan says that Margaret threatened him and said they would kill him if he did not sign it. Margaret then pushed Mr. Dogan back who fell on his bed.

[80] Margaret Pakulski and Joanne Kalaj were added as defendants and served with the amended statement of claim. They were noted in default and thus are deemed to admit the truth of the allegations of fact made in the amended statement of claim. That pleading alleges, amongst other things, that the defendants attended at the plaintiff's residence and screamed obscenities at him and threatened the plaintiff's physical security and threatened to poison him and that Margaret Pakulski on or about September 2, 2004 attended at the plaintiff's premises and physically assaulted him.

[81] On October 1, 2004 Mr. Dogan came home to his premises after being at the bank. He found that all his belongings had been dumped out of the room and that his room was locked. Margaret Pakulski, Joanne Kalaj and one of their brothers was there at the time. The police were called. Mr. Dogan took his belongings to a friend's house. His friend took him to the hospital. His blood pressure and heart beat was highly elevated and could not be brought down until midnight. He was kept in the hospital for three days.

[82] The amended statement of claim pleads that the defendants entered the plaintiff's premises without his consent and without prior notice and removed the grill, refrigerator, and dining room table which they had installed for his use, that they removed his personal items and that they changed the locks without notice. It pleads that they took his rental funds and after illegally locking him out of the premises, failed to return his money. The rent of \$350 was not returned.

[83] The amended statement of claim further pleads that as the result of the conduct pleaded, the plaintiff's physical and mental health deteriorated, his medical conditions were worsened, he has undergone financial and mental stress and he has required treatment for his physical and physiological health. It states that the defendants knew or ought to have known that their

conduct would cause the plaintiff physical and/or physiological distress. Margaret Pakulski and Joanne Kalaj are deemed to admit the truth of those allegations of fact.

Liability for Tortious Conduct

(a) Battery

[84] A person who intentionally causes a harmful or offensive contact with another person is liable for battery even if no actual harm has been caused to a plaintiff. *Linden, Canadian Tort Law*, Eighth Edition states at pages 43-44:

A person who intentionally causes a harmful or offensive contact with another person is liable for battery. This nominate tort protects the interest in bodily security from deliberate interferences by others. It seeks to reduce the incidence of violence in our society.

Nor is it necessary that a battery cause any actual harm to the plaintiff. Offensive contact is enough, however trivial it may seem, for it may trigger retaliatory measures by persons whose dignity and self-respect are threatened thereby. Better to permit an action for these seemingly minor intrusions than to invite violent counter-attacks by the aggrieved victims.

[85] It is clear that Margaret Pakulski committed battery on Mr. Dogan and is liable for that battery.

(b) Assault

[86] Assault is the intentional creation of the apprehension of imminent harmful or offensive contact. *Linden, Canadian Tort Law*, Eighth Edition states at pages 46-47:

Assault is the intentional creation of the apprehension of imminent harmful or offensive contact. The tort of assault furnishes protection for the interest in freedom from fear of being physically interfered with. Damages are recoverable by someone who is made apprehensive of immediate physical contact, even though that contact never actually occurs. The underlying policy thrust of the tort of assault, like that of battery, is the reduction of violence. Because threatening to inflict harm is apt to attract retaliation in the same way as causing harm, it must also be discouraged by tort law.

Conduct which intentionally arouses apprehension of an imminent battery constitutes an assault. Shaking a fist at another person, lunging at someone in an effort to attack, and swinging an axe at another person, are actionable assaults. An assault may also be committed if a person is surrounded by a group of people in a hostile manner, ...

[87] Both Margaret Pakulski and Joanne Kalaj are liable for the tort of assault for their actions when they and Margaret Pakulski's boyfriend went to Mr. Dogan's premises on the occasion when Margaret Pakulski assaulted Mr. Dogan. Their actions that day were clearly taken in concert and they were taken to threaten Mr. Dogan's physical security. The addition of Margaret Pakulski's boyfriend on the scene can only have been for the purpose of intimidating Mr. Dogan and adding to the threat of the imminence of physical harm if Mr. Dogan did not sign the document that they wanted him to sign.

(c) **Eviction**

[88] There is no evidence that the defendants did not have the right to evict Mr. Dogan as a matter of landlord and tenant law, nor was I provided with any authorities to suggest otherwise. However, it was clearly a breach of Mr. Dogan's rights to be evicted and not to have rent that he had prepaid repaid to him. It is also clear that the eviction was the combination of the course of conduct that began in late August or early September 2004 as a result of Mr. Dogan refusing to release his claim for payment of a mere \$500. The action in evicting Mr. Dogan was high-handed and abusive and, as pleaded, made his medical condition worse and caused him some mental distress and hospitalization.

(d) **Damages**

[89] The battery by Margaret Pakulski did not in itself cause any physical injury to Mr. Dogan and, while actual harm is not required to constitute the tort of battery, that fact should be taken into account in assessing damages. The battery, however, was a cause of distress to Mr. Dogan and worsened his medical conditions and stress. I assess the damages caused by Margaret Pakulski's battery at \$1,000.

[90] The assault by Margaret Pakulski and Joanne Kalaj that occurred at the time of the battery by Margaret Pakulski was serious, and intended to be so. I assess the damages caused by the assault at \$1,000.

(e) **Aggravated and Punitive Damages**

[91] *Linden, Canadian Tort Law*, Eight Edition states at page 65-66:

Aggravated and punitive damages may be distinguished from one another according to their purpose. Aggravated damages are intended to compensate a plaintiff whose injuries have been exacerbated by particularly outrageous conduct by the defendant. Punitive damages are intended to punish the defendants and to deter others. Aggravated and punitive damages are easily confused with one another because in each case the award is triggered by exceptionally objectionable conduct by the defendant.

Aggravated damages are also compensatory, but may be awarded only in cases where a defendant's conduct has been "particularly high-handed or oppressive, thereby increasing the plaintiff's humiliation or anxiety". To allow such damages there must be a finding that a defendant was "motivated by actual malice, which increased the injury to the plaintiff . . . by increasing the mental distress and humiliation of the plaintiff". Aggravated damages express the "natural indignation of right-thinking people arising from the malicious conduct of the defendant". Mr. Justice La Forest has recently explained that "[a]ggravated damages may be awarded if the battery has occurred in humiliating or undignified circumstances".

Even though the same sort of egregious conduct triggers them, punitive or exemplary damages, unlike general and aggravated damages, are not compensatory. Their aim is "to punish" a defendant and to express "outrage at the egregious conduct of the defendant". They are akin to a civil fine that is meant to "act as a deterrent to the defendant and to others from acting in this manner". Exemplary damages may be awarded only "where the combined award of general and aggravated damages would be insufficient to achieve the goal of punishment and deterrence". Exemplary damages are necessary, for, according to Cory J., without them the wealthy and powerful might regard awards of general damages as a "license fee" to continue harming "vulnerable victims".

[92] The conduct of Margaret Pakulski and Joanne Kalaj was particularly high-handed and oppressive and intended to increase Mr. Dogan's humiliation and anxiety. It had the effect of

worsening his mental and medical conditions. Mr. Dogan is entitled to aggravated damages against Margaret Pakulski and Joanne Kalaj which I assess at \$2,500.

[93] In my view punitive damages are warranted in this case. The combined award of general and aggravated damages is not sufficient to achieve the goal of punishment and deterrence. The actions of Margaret Pakulski and Joanne Kalaj were visited upon a vulnerable victim and they were callous in the extreme. I assess punitive damages against Margaret Pakulski and Joanne Kalaj at \$2,500.

[94] In summary, Margaret Pakulski is liable for \$1,000 for battery. Margaret Pakulski and Joanne Kalaj are jointly and severally liable for \$1,000 for assault, \$2,500 for aggravated damages and \$2,500 for punitive damages. Mr. and Mrs. Pakulski are liable to repay Mr. Dogan the \$350 for rent he had prepaid for the period that he was evicted.

Costs

[95] This action was not commenced under the Simplified Procedure provided in Rule 76. Rule 76.13 (3) provides that in the circumstances the plaintiff shall not recover any costs unless the court is satisfied that it was reasonable for the plaintiff to have commenced and continued the action under the ordinary procedure.

[96] Ms Nelson submits that it was reasonable to start the action under the ordinary procedures. She says that Mr. Dogan had suffered a head injury at the time of the statement of claim and it was unknown to what extent that injury would be. There was also a possibility of MS. She further submits that the only step that was taken beyond the Simplified Procedures provided for in Rule 76 was the examination for discovery of Margaret Pakulski and Joanne Kalaj.

[97] This action was commenced on August 5, 2004, some 16 months after the fall. General damages of \$500,000 and special damages of \$500,000 were claimed. On November 18, 2004, the claim was amended to claim a further \$500,000 for punitive damages and \$500,000 for aggravated damages. It had to be obvious by then that the amounts claimed were not achievable.

However, I cannot say that the damages of \$50,000 was so out of the question that the action should have been commenced or continued under the Simplified Procedure rule.

[98] Ms Nelson has in effect proceeded with the case as a simplified matter, with the exception of conducting examinations for discovery of the defendant Margaret Pakulski and Joanne Kalaj. I would not order any costs for those examinations. The trial was short, and while it took place sporadically over three days, it was really a one and a half day trial. I award the plaintiff costs of \$3,000 against the defendants.

NEWBOULD J.

Released: May 11, 2007

COURT FILE NO.: 04-CV-273613CM3
DATE: 20070511

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

MECIT DOGAN

Plaintiff

- and -

LESTER PAKULSKI, WANDA PAKULSKI,
MARGARET PAKULSKI and JOANNE
PAKULSKI, also known as JOANNE KALAJ

Defendants

REASONS FOR JUDGMENT

NEWBOULD J.

Released: May 11, 2007